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7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **MR.DINKAR SAWANT**	**Age :** 71 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. UJJWALA**	**Registered on** : 31-08-2026 02:02 PM	
Reg No	: **HL4001832088 / HYMH/BVI/MH00022**	**Received on** : 31-08-2026 05:57 PM	
Barcode	: **HT11314448**	**Reported on** : 01-09-2026 01:26 PM	
INV	: AMMONIA		**SAMPLE :** Heparin

AMMONIA

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
AMMONIA Method: Spectrophotometry	87.6	umol/L	. - .

Reference Range

- 170-340 umol/L in new-borns,
- 70-135 umol/L in children and
- 15-60 umol/L in adults

Clinical significance :

Ammonia is metabolized exclusively in the liver through the urea cycle enzymes which convert ammonia into urea. Toxic build up of ammonia due to disruption of this metabolism produces neurotoxic effects. Elevated serum levels of ammonia indicate some form of liver failure. Increased levels * Congenital deficiency of Urea cycle enzymes * Cirrhosis * Acute fulminant hepatitis * Reye's syndrome * Hepatic encephalopathy

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)

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REPORT

PT Name	: **SNEHAL SUPNWAR**	**Age :** 21 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. VISHNUDAS KHANDADE**	**Registered on** : 26-08-2026 06:53 PM	SHRI CLINICAL LABORATORY, BARURE HOSPITAL LATUR
Reg No	: **HL8501827691 / MH451**	**Received on** : 27-08-2026 10:17 AM	
Barcode	: **HEL148676**	**Reported on** : 29-08-2026 05:08 PM	**SAMPLE :** Serum
INV	: ANTIMOG (MYELIN OLIGODENDROCYTE GLYCOPROTEIN)		

ANTIMOG (MYELIN OLIGODENDROCYTE GLYCOPROTEIN)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Anti-MOG (MYELIN OLIGODENDROCYTE GLYCOPROTEIN) ANTIBODY, SERUM *	NEGATIVE	.	. - .
Method: (Cell based assay, IFA)			

NOTE

1. Auto Immune reactivity are not by themselves diagnostic and must be correlated with other laboratory and clinical findings.
2. Test conducted on 1:10 diluted serum.
3. Antigen substrate - transfected cells of EU-90.

Comments

NMO (Neuromyelitis optica) is a chronic autoimmune disease of CNS with different manifestations (NMO spectrum disease; NMOSD). Antibodies against water channel aquaporin-4 which is expressed on astrocytes, are established biomarkers for NMOSD and are detectable in 60% to 90% of patients. In 7.4% of cases, however, antibodies against MOG are also produced. In NMOSD patient who are anti-aquaporin-4 negative, MOG antibodies occur in 21.1% of sera. Determination of autoantibodies against MOG is especially relevant for the diagnosis of acquired demyelinating diseases of the CNS, particularly in children, and NMO in adults. 30%-40% of patients with inflammatory demyelinating disease of the CNS show antibodies against MOG at disease onset.

IMPORTANT INSTRUCTIONS

*Test results released pertain to the specimen submitted .
*All test results are dependent on the quality of the sample received by the Laboratory .
*Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician .
*Sample repeats are accepted on request of Referring Physician within 7 days post reporting.
*Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted.
*Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting.
*Test results may show interlaboratory variations.
*The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s).
*Test results are not valid for medico legal purposes.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **JAMAL AHMED ANSARI**	**Age :** 64 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 22-08-2026 08:19 PM	MH CLINICAL LAB
Reg No	: **HL7001823114 / RAJ41/RAY03**	**Received on** : 22-08-2026 08:39 PM	
Barcode	: **HEN116261**	**Reported on** : 24-08-2026 12:58 PM	
INV	: ANTI MuSK (MUSCLE SPECIFIC KINASE) ANTIBODY		**SAMPLE :** Serum

ANTI MUSK (MUSCLE SPECIFIC KINASE) ANTIBODY

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
MuSK (MUSCLE SPECIFIC KINASE) ANTIBODY	0.39	U/mL	0.00 - 1.00

Method: Radioimmunoassay (RIA)

Clinical significance :

Fatigable weakness due to impaired synaptic transmission at the neuromuscular junction is characteristic of myasthenia gravis (MG). The diagnosis is made by clinical and electromyographic criteria. Positive autoimmune serology must be interpreted in the clinical and electrophysiological context and response to anticholinesterase medication. Most cases are autoimmune and are caused by IgG autoantibodies binding to critical postsynaptic membrane molecules (nicotinic acetylcholine receptor or its interacting proteins).(1) Autoantibody detection frequency is lowest in patients with weakness confined to extraocular muscles (71% muscle acetylcholine receptor: AChR binding).(2) Mayo Clinic Laboratories' first-line serological evaluation detects muscle AChR antibody in 92% of nonimmunosuppressed patients with generalized weakness due to MG. Muscle-specific kinase (MuSK) antibody is detectable in more than one-third of those seronegative for muscle AChR antibody (less than 4% of all patients).(3) Physiologically, MuSK is involved in integrating and stabilizing AChR clusters in the motor endplate. MuSK is activated when the nerve-derived proteoglycan agrin binds to its receptor, lipoprotein-related protein 4 (LRP4). Antibodies to LRP4 itself have been described in rare patients.(1)

Six percent of nonimmunosuppressed patients with generalized MG lack demonstrable AChR or MuSK antibodies (double seronegative). Other rare autoantibodies no doubt remain to be discovered in such cases. However, as in autoimmune AChR MG and MuSK MG, testing for common organ-specific and nonorgan-specific autoantibodies is a valuable ancillary investigation in evaluating seronegative acquired generalized MG. General serological testing, coupled with family or personal history, will disclose autoimmune phenomena in 77% of those cases.(3) These disorders may include thyroid disease, type 1 diabetes, vitiligo, premature greying, rheumatoid arthritis, or lupus. Testing may also reveal antinuclear antibodies, glutamic acid decarboxylase (GAD65) antibodies, thyroperoxidase/thyroglobulin antibodies, or gastric parietal cell antibodies.(3) Objective improvement in strength following a therapeutic trial of plasmapheresis or intravenous immune globulin would justify consideration of long-term immunosuppression.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **VISHWANATH RAKSHE**
Ref By : **DR. PARASNATH CLINICS**
Reg No : **HL9101830226 / MH080**
Barcode : **hej068413**
INV : BLEEDING TIME AND CLOTTING TIME (BT, CT)

Age : 39 Year | **Sex :** Male
Registered on : 29-08-2026 02:19 PM
Received on : 29-08-2026 04:49 PM
Reported on : 29-08-2026 06:22 PM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

BLEEDING TIME AND CLOTTING TIME (BT, CT)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Bleeding Time Method: .	3.20	MIN&SEC	2.5 - 9.5
Cloitting Time Method: .	5.40	MIN&SEC	4.0 - 12.0

Clinical significance :

Most people will never need a bleeding time test. You may need to have a bleeding test if you've been experiencing bleeding that won't stop, especially from small incisions, punctures, or cuts.
Your doctor can choose from a number of tests to evaluate your platelet function. A bleeding time test is a common test to screen patients having prolonged bleeding times.
Abnormal results from a bleeding time test can be a sign that you need more in-depth testing to find the cause of your prolonged bleeding. It could mean you have an acquired platelet function defect, which is a condition that develops after birth and affects how well your blood platelets work. Your body may produce too many or too few platelets, or your platelets may not work properly.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MRS SONAL**	**Age :** 33 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** JAY PATH LABORATORY
Ref By	: **DR. SIDHARTH PATEL**	**Registered on :** 29-08-2026 02:20 PM	
Reg No	: **HL2101830222 / GUJ016**	**Received on** : 30-08-2026 09:02 AM	
Barcode	: **HEN183021**	**Reported on** : 30-08-2026 12:12 PM	
INV	: Chikungunya IgG & IgM		**SAMPLE :** Serum

CHIKUNGUNYA IGG & IGM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Chikungunya - IgG Method: ELISA	0.12	ODRatio	0.00 - 1.00
Chikungunya - IgM Method: ELISA	0.32	ODRatio	0.00 - 1.00

Clinical significance :

IgM and IgG Negative:
-No serologic evidence of exposure to Chikungunya virus. Repeat testing on a new specimen collected in 5 to 10 days is recommended if clinical suspicion persists.

IgM and IgG Positive:
-IgM and IgG antibodies to Chikungunya virus detected, suggesting recent or past infection. IgM antibodies to Chikungunya virus may remain detectable for 3 to 4 months post-infection.

IgM Positive, IgG Negative:
-IgM antibodies to Chikungunya virus detected, suggesting recent infection. Repeat testing in 5 to 10 days is recommended to demonstrate anti-Chikungunya virus IgG seroconversion to confirm current infection.

IgM Negative, IgG Positive:
-IgG antibodies to Chikungunya virus detected, suggesting past infection.

IgM and/or IgG Borderline:
-Repeat testing in 10 to 14 days is recommended.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **MR RAMAN JAGTAP**
Ref By : **DR. J.J.JAIN**
Reg No : **HL9701832201 / MH090**
Barcode : **HEN178804**
INV : Chikungunya-IgM

Age : 32 Year | **Sex :** Male
Registered on : 31-08-2026 02:58 PM
Received on : 31-08-2026 06:32 PM
Reported on : 31-08-2026 09:37 PM

SAMPLE COLLECTED AT :

SAMPLE : Serum

CHIKUNGUNYA-IGM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Chikungunya - IgM Method: ELISA	0.32	OD Ratio	0.0 - 1.0

Clinical significance :

IgM Negative:
-No serologic evidence of exposure to Chikungunya virus. Repeat testing on a new specimen collected in 5 to 10 days is recommended if clinical suspicion persists.

IgM Positive:
-IgM antibodies to Chikungunya virus detected, suggesting recent or past infection. IgM antibodies to Chikungunya virus may remain detectable for 3 to 4 months post-infection.

IgM Positive, :
-IgM antibodies to Chikungunya virus detected, suggesting recent infection. Repeat testing in 5 to 10 days is recommended to demonstrate anti-Chikungunya virus IgM seroconversion to confirm current infection.

IgM Negative,
-IgM antibodies to Chikungunya virus detected, suggesting past infection.

IgM Borderline:
-Repeat testing in 10 to 14 days is recommended.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **ARMAN**
Ref By : **DR. SELF**
Reg No : **HL3301824561 / MH024**
Barcode : **HEN137281**
INV : DRUGS OF ABUSE (10 Drugs)

Age : 21 Year | **Sex :** Male
Registered on : 24-08-2026 02:23 PM
Received on : 25-08-2026 05:49 PM
Reported on : 26-08-2026 04:37 PM

SAMPLE COLLECTED AT :
AK DIAGNOSTIC CENTER

SAMPLE : Urine

DRUGS OF ABUSE (10 DRUGS)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Amphetamine	NEGATIVE	.	. - .
Benzodiazepine	NEGATIVE	.	. - .
Barbiturates	NEGATIVE	.	. - .
Morphine	NEGATIVE	.	. - .
Cocaine	NEGATIVE	.	. - .
Tetrahydrocannabinol (THC)	POSITIVE	.	. - .
Marijuana Method: -	NEGATIVE	-	- - -
Opiate Method: -	NEGATIVE	-	- - -
Methadone Method: -	NEGATIVE	-	- - -
Fentanyl Method: -	NEGATIVE	-	- - -

Clinical significance :
The drugs that may be addictive target your brain's reward system. They flood your brain with a chemical called dopamine. This triggers a feeling of intense pleasure. So you keep taking the drug to chase that high.
Over time, your brain gets used to the extra dopamine. So you might need to take more of the drug to get the same good feeling. And other things you enjoyed, like food and hanging out with family, may give you less pleasure.
When you use drugs for a long time, it can cause changes in other brain chemical systems and circuits as well. They can hurt your:

Please correlate with clinical conditions

~~End of report~~

Dr. MOUSHMI MUKHERJEE (MD PATH)
Director (Reg. 61873)

Dr. MEENU RANI (MD PATH)
Consultant Pathologist

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REPORT

PT Name : **SALIMA TAMBOLI**
Ref By : **DR. NITIN PHADKE**
Reg No : **HL6801825820 / MH602**
Barcode : **HEN123741**
INV : FAECAL CALPROTECTIN OR FECAL CALPROTECTIN

Age : 60 Year | **Sex :** Female
Registered on : 25-08-2026 01:01 PM
Received on : 25-08-2026 03:48 PM
Reported on : 27-08-2026 03:05 PM

SAMPLE COLLECTED AT :
SNS PATHO LAB

SAMPLE : STOOL

FAECAL CALPROTECTIN OR FECAL CALPROTECTIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
FECAL CALPROTECTIN Method: CLIA	46.5	ug/g	0.0 - 50.0

Interpretation :

RESULT in µg/g feces	Remarks
<50.00	Negative
50.00 - 120.00	Borderline
>120.00	Positive

Clinical significance :

The primary goal of this study will be to assess whether stool collected and frozen from anonymous screened unrelated donors can be as effective as stool freshly collected from recipient's parents when used in Fecal Microbial Transplant for the eradication of recurrent Clostridium difficile infections in children. In the current protocols, which are more than 90% effective, each child who is receiving a fecal transplant has to provide their own donor stool, usually from a parent or close relative. This requires considerable screening costs for each case and is logistically complicated as the donor must be present and must stool just prior to the transplant. The investigators hope to show that a small number of healthy donors can provide stool samples which can be frozen and banked and then thawed for use in numerous patients. The primary goal is to show that Clostridium difficile will be eradicated as effectively (Greater than 90% success) when using the stool from the frozen donors.
The study will also evaluate the inflammatory response and intestinal microbiome in young children aged 1-3 years with Clostridium difficile infections to better predict which ones will respond to fecal transplantation and which ones have incidental infections. For this question the investigators will gather stool samples to check for lactoferrin, calprotectin, and alpha1antitrypsin, and 16s ribosomal RNA analysis in children before and after the fecal transplants. The goal is to see if there is an intestinal microbiome that predisposes some children to getting sick from Clostridium difficile versus just having it incidentally.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name : **SANA**
Ref By : **DR. SELF**
Reg No : **HL9701832573 / RAJ41**
Barcode : **HEL002901**
INV : Basic Fever Panel

Age : 30 Year | **Sex :** Female
Registered on : 31-08-2026 11:40 PM
Received on : 01-09-2026 04:10 AM
Reported on : 01-09-2026 10:47 AM

SAMPLE COLLECTED AT :

SAMPLE : EDTA Blood

MALARIAL PARASITE SLIDE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Giemsa Stain Method: Giemsa Stain	NOT SEEN	.	. - .

Clinical Information

Malaria is caused primarily by 4 species of the protozoa *Plasmodium*: *P falciparum*, *P vivax*, *P malariae*, and *P ovale.* A fifth *Plasmodium* species, *P knowlesi*, is a simian parasite that may be an important source of human infection in some regions of Southeast Asia. Differentiating *P falciparum* and *P knowlesi* from other species is important since both can cause life-threatening infections. In addition, *P falciparum* is typically resistant to many commonly used antimalarial agents such as chloroquine.

Babesiosis is an emergent zoonosis caused by an intraerythrocytic protozoan in the genus *Babesia. Babesia microti* is responsible for the vast majority of human cases in the United States, with "hot spots" of disease along the Northeast Coast (eg, Martha's Vineyard, Long Island, and Nantucket) and Midwest states, although the distribution of disease is spreading. In addition, a small number of cases of *Babesia duncani* and *Babesia duncani*-like human infection (WA and CA strains) have been reported along Pacific Coast states from Washington to northern California, and *Babesia divergens/B divergens*-like strains have been isolated from humans in Missouri (MO-1 strain), Kentucky, and Washington. At this time, only *Babesia microti* is a nationally notifiable disease.

Babesia microti shares a tick vector with *Borrelia burgdorferi* and *Anaplasma phagocytophilum*, the causative agents of Lyme disease and human granulocytic anaplasmosis (HGA), respectively. Recent studies suggest that exposure to *Babesia microti* is quite common in areas endemic for Lyme disease and anaplasmosis, so it is prudent to consider testing for all 3 diseases concurrently. Less commonly, babesiosis may be acquired through blood transfusion, and therefore the FDA approved testing for this parasite in donor units in 2018.

Most patients with babesiosis have a mild illness or are asymptomatic, but some develop a severe illness that may result in death. Patient symptoms may include fever, chills, extreme fatigue, and severe anemia. The most severe cases occur in asplenic individuals and those over 50 years of age. Rare cases of chronic parasitemia, usually in immunocompromised patients, have been described.

Microscopy of Giemsa-stained thick and thin blood films is the standard laboratory method for diagnosis and differentiation of *Plasmodium* and *Babesia* species. Under optimal conditions, the sensitivity of the thick film microscopy is estimated to be 10 to 30 parasites per microliter of blood. This test can also detect trypanosomes that cause Chagas disease (*Trypanosoma cruzi*) and African sleeping sickness (*T brucei*), as well as some species of filariae. If filarial infection is suspected, FIL / Filaria, Blood is recommended since it is more sensitive than the traditional blood smear examination.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist

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REPORT

PT Name	: **KOMAL SURAJ GAWADE**	**Age :** 27 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. MAYUR MORE**	**Registered on** : 30-08-2026 04:23 PM	
Reg No	: **HL6401831428 / MH428**	**Received on** : 30-08-2026 07:56 PM	
Barcode	: **HEJ067572**	**Reported on** : 30-08-2026 10:25 PM	
INV	: ORAL GLUCOSE CHALLENGE TEST (OGCT)		**SAMPLE :** Fluoride Plasma-01

ORAL GLUCOSE CHALLENGE TEST (OGCT)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
OGCT	99.30	mg/dL	0 - 140.0
Method: GOD-POD			

Clinical significance :

The glucose challenge test measures your body's response to sugar (glucose). The glucose challenge test is done during pregnancy to screen for gestational diabetes — diabetes that develops during pregnancy.

The glucose challenge test is done in two steps. First you drink a sugary solution. One hour later, your blood sugar level is measured. The results of the glucose challenge test indicate whether you might have gestational diabetes.

If the test results are above normal, you'll need to have further testing to determine the diagnosis.

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **MR.YAQOOB**	**Age :** 37 Y | **Sex :** Male	**SAMPLE COLLECTED AT :** PULS DIAGNOSTIC CENTRE - 7718915222
Ref By	: **DR. ZAFFAR SAYYED**	**Registered on :** 25-08-2026 11:23 AM	
Reg No	: **HL5001825614 / RAJ31**	**Received on** : 25-08-2026 03:13 PM	
Barcode	: **HEN121603**	**Reported on** : 27-08-2026 05:16 PM	
INV	: SEMEN C/S		**SAMPLE :** SEMEN

SEMEN C/S

TEST DESCRIPTION	RESULT
RESULT Method: DISC. DIFFUSION	No growth
Incubation period	obtained after 24 / 48 hrs of incubation at 37°c
Colony count	.

REMARKS :
Condition which can cause a false negative Cultures :

1. Patient is on Antibiotics.
2. Anaerobic bacteria.
3. Fastidious aerobic bacteria which are not able to grow on routine culture media.
4. Besides all these factors, at least in 25-40 % of cases there is on direct correlation between in vivo clinincal pictures and in-vitro culture result.

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Dr. AMIT AGRAWAL (MD MICROBIOLOGY)
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REPORT

PT Name	: **CHANDRAVATI DEVI**	**Age :** 58 Year | **Sex :** Female	**SAMPLE COLLECTED AT :** RX PATHOLOGY
Ref By	: **DR. SATRUPA DEB**	**Registered on :** 30-08-2026 01:10 PM	
Reg No	: **HL5401831158 / RAJ40**	**Received on** : 30-08-2026 04:20 PM	
Barcode	: **HED058411**	**Reported on** : 30-08-2026 08:38 PM	
INV	: Serum Phosphate		**SAMPLE :** Serum

SERUM PHOSPHATE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
serum phosphate Method: Molybdate UV	4.80	mg/dl	2.5 - 5.0

Clinical significance :

Phosphate, or phosphorous, is similar to calcium, as it is found in your teeth and bones. Like calcium, you need vitamin D in order to absorb phosphate properly. The kidneys excrete phosphate. Therefore, the most common cause of increased phosphate levels (or hyperphosphatemia) is the kidney's inability to get rid of phosphate. Hyperphosphatemia is also seen in people who have:

- Excessive dietary intake of phosphate (also from laxatives or enemas)
- Your body may have a deficiency in calcium or magnesium, or it may have too much Vitamin D, resulting in hyperphosphatemia.
- Severe infections can cause increased phosphate levels, resulting in hyperphosphatemia.
- Cell destruction - from chemotherapy, when the tumor cells die at a fast rate. This can cause tumor lysis syndrome.
- Problems with your breathing (respiratory acidosis)
- You may have high phosphate levels from prolonged exercise, which causes muscle damage. Certain athletes and distance runners may get this, called rhabdomyolysis.
- You may have problems with your thyroid, parathyroid gland, or other hormones, causing increased levels of phosphate in your blood and resulting in hyperphosphatemia
- Kidney failure can cause hyperphosphatemia

Please correlate with clinical conditions

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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Consultant Pathologist (Reg. 0375)

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HY PATHO LAB MUMBAI

Corporate Office : HY PATHO LAB
CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028
7718962488 info@hypatholab.in www.hypatholab.in Toll Free No. : 18001030287

REPORT

PT Name	: **MRS LOTUS ARORA**	**Age :** 60 Year | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on** : 21-08-2026 02:53 PM	
Reg No	: **HL3901821662 / RAJ47**	**Received on** : 21-08-2026 05:15 PM	
Barcode	: **HDR218393**	**Reported on** : 22-08-2026 11:55 AM	**SAMPLE :** Serum
INV	: Tissue Transglutaminase Antibody (TTG-IgM)		

TISSUE TRANSGLUTAMINASE ANTIBODY (TTG-IGM)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Tissue Transglutaminase Antibody - IgM	5.20	U/mL	. - .

Normal Range:
< 20.0 : Negative
20.0-30.0 : Weak Positive
> 30.0 : Positive

Clinical significance :
Presence of the tissue transglutaminase (tTG) IgM antibody is associated with glutensensitive enteropathies such as celiac disease and dermatitis herpetiformis. tTG IgM antibody concentrations greater than 30 U/mL usually correlate with results of duodenal biopsies consistent with a diagnosis of celiac disease. For antibody concentrations greater or equal to 4 U/mL but less than or equal to 30 U/mL, additional testing for endomysial (EMA) IgA concentrations may improve the positive predictive value for disease.

~~End of report~~

Dr. NEHAL DATTARAM RAHATE (MD PATH)
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REPORT

PT Name	: **KAMINI SHIKARE.**	**Age :** 35 Y | **Sex :** Female	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SANTOSH BHOSEKAR**	**Registered on :** 21-08-2026 08:09 PM	
Reg No	: **HL1601822042 / MH207**	**Received on** : 22-08-2026 09:33 AM	
Barcode	: **HEN109144**	**Reported on** : 24-08-2026 03:26 PM	**SAMPLE :** SWAB
INV	: VAGINAL SWAB C/S		

VAGINAL SWAB C/S

TEST DESCRIPTION	RESULT
Results Method: Disc Diffusion	No growth
Incubation period Method: .	obtained after 24 / 48 hrs of incubation at 37°c
Colony count Method: .	.

REMARKS :
Condition which can cause a false negative Cultures :

1. Patient is on Antibiotics.
2. Anaerobic bacteria.
3. Fastidious aerobic bacteria which are not able to grow on routine culture media.
4. Besides all these factors, at least in 25-40 % of cases there is on direct correlation between in vivo clinincal pictures and in-vitro culture result.

Interpretation

INTERPRETATION
(++++) or (+++) : maximum sensitivity
(++) : moderate sensitivity
(+) : minimum sensitivity
(R) : resistant

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Consultant Microbiology

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REPORT

PT Name	: **MR HASAN PATHAN**	**Age :** 60 Year | **Sex :** Male	**SAMPLE COLLECTED AT :**
Ref By	: **DR. SELF**	**Registered on :** 31-08-2026 12:04 AM	
Reg No	: **HL1501831619 / MH004**	**Received on** : 31-08-2026 08:38 AM	
Barcode	: **HEJ089035**	**Reported on** : 01-09-2026 12:17 PM	**SAMPLE :** Serum
INV	: Widal Test (Tube Test)		

WIDAL TEST (TUBE TEST)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Salmonella typhi O (TO) Method: Tube Agglutination	1:80	.	. - . < 1 : 80 NEGATIVE
Salmonella typhi H (TH) Method: Tube Agglutination	1:40	.	. - . < 1 : 80 NEGATIVE
Salmonella paratyphi A, H (AH) Method: Tube Agglutination	1:20	.	. - . < 1 : 80 NEGATIVE
Salmonella paratyphi B, H (BH) Method: Tube Agglutination	1:20	.	. - . < 1 : 80 NEGATIVE
RESULT Method: .	NEGATIVE	.	. - .

Interpretation :

* The sample which shows the titre of above 1:80 for O agglutinations and above 1:160 for H agglutination should be considered as clinically significant (active infection).
* Demonstration of 4-fold rise between the two is diagnostic.
* H agglutination is more reliable than O agglutinin.

Limitations :

* It is a presumptive serological test for enteric fever.
* In low typhoid endemic areas, weak and delayed O and H antibody responses limit the usefulness of the Widal test.
* The Widal test should be interpreted in the light of baseline titers in a healthy local population

~~End of report~~

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